

****Informal Hearing of the President of the Authority for the Protection of Personal Data, Prof. Pasquale Stanzione - Examination of Resolutions Loizzo No. 7-00183 and Girelli No. 7-00183, Regarding the Collection and Use of Health Data****

Chamber of Deputies - XII Committee on Social Affairs

(13 February 2024)

I thank you for this invitation, which allows for a timely discussion on such a relevant topic as that addressed by Resolutions Girelli and Loizzo. These resolutions tackle, from substantially similar perspectives, the issue of the relationship between privacy and health, which must be framed within the broader balance between health and the dignity of the individual, as underscored by Article 32 of the Constitution.

The resolutions commit the Government, in summary, to enhance the circulation of health data for research, care, and clinical governance purposes. This applies both at the national level—promoting already initiated digitalization pathways in healthcare, which are still susceptible to expansion—and at the European level, supporting the approval of the Regulation on the European Health Data Space. The Authority can only endorse this objective, while obviously emphasizing the need to align every step of this process with data protection regulations.

As the resolutions highlight, it is indeed essential to protect our data, particularly health data, which has an extraordinarily ambivalent nature. They are, in fact, a valuable tool for guaranteeing the right to health and care (which, with foresight, our Constitution ensures even “for the needy”), including in the solidarity component of their allocation for research purposes. At the same time, they are a precious fragment of each individual's most intimate life, which must be protected from undue interference or exploitation. It is no coincidence that among the first norms of the legal framework on privacy are those concerning health data, which must be protected to prevent breaches of diagnosis and therapy, thereby building, based on the trust placed in professional secrecy, that strictly fiduciary relationship between doctor and patient that constitutes the cornerstone of today's regulations and jurisprudence on therapeutic self-determination.

This is particularly true due to the absolutely peculiar nature of health data: hypersensitive, as we would have said years ago, because they express the most authentic essence of privacy: of the body, its pathologies, its deficiencies, its irregularities, and for this reason, they are susceptible to exposing individuals to the most petty discriminations, stigmatizations, and classifications.

Not surprisingly, health data are among those that benefit from the greatest protection granted by the legal framework: strengthened guarantee measures, particularly stringent conditions for the lawfulness of processing, a more stringent interpretation of the principle of proportionality, equal rank as a strict criterion for legitimizing access even to mere documentation, and strict necessity for informational purposes as a parameter for the admissibility of journalistic communication, along with a prohibition on disclosure.

These guarantees must also be ensured—more so—in a context of increasingly marked digitalization of healthcare, which must, however, be framed within a comprehensive and forward-looking health governance project. This must, in particular, minimize cyber risks and promote selective data sharing, allowing for (as provided by the Regulation on the European Health Data Space) solidarity-based allocation, even for research purposes, but with the necessary precautions to avoid any undue re-identification of the data subjects and discrimination against groups (a risk that is all the more elevated when the datasets on which the algorithmic decision is based are not representative or are nonetheless underpinned by biases related to gender, ethnicity, social conditions, or indeed health, etc.).

The protection of personal data is, in this sense, an essential component of this process, representing both a guarantee for the individual and a factor of appropriateness in health governance and citizen trust in healthcare.

There is indeed a pressing need to protect healthcare systems that, the more computerized and

interoperable they become, appear increasingly vulnerable, exposed to cyberattacks that are more extensive than ever in this sector. The attack launched through computer systems on healthcare networks can have such devastating effects that it can lead to (as in the case in the United States a few years ago) the paralysis of entire healthcare facilities, which must therefore be protected, especially in their technological infrastructure.

Moreover, poorly governed technology can exponentially increase the clinical risk, reflecting the cybersecurity risk, especially if, for example, the data on which the diagnosis is based are altered. This is a type of risk that is becoming increasingly frequent with the use of big data, artificial intelligence systems, and algorithms, which, if “trained” with incorrect data or non-representative statistical series, risk leading to incorrect diagnoses and sometimes even to outright discrimination, worse than that from which computational power was supposed to free us.

The accuracy of the data used in configuring algorithms indeed determines the “intelligence” and fairness of their choices. If the classification of the reference cases provided to the algorithm to decide, for example, the nature of a pathology or to evaluate a marker is incorrect, then the resulting diagnosis will be...

wrong, with potentially fatal effects for the patient. The quality and accuracy of the data (essential principles of the GDPR) are, in this sense, a prerequisite for the effectiveness of big data analytics, especially in such a sensitive sector as healthcare.

In the Decalogue on the use of artificial intelligence in healthcare, adopted by the Authority last October, the principles governing the correct use of personal data through AI systems are recalled, which can be summarized in the principles of transparency and oversight of automated decision-making processes, as well as non-algorithmic discrimination.

At present, the processing of data carried out through the Electronic Health Record (EHR) does not foresee the use of AI systems, which should be the subject of a regulatory provision to be submitted for the Authority's opinion and including adequate guarantees for patients. A similar regulatory provision would be necessary for the inclusion, in the EHR—proposed by the resolutions—of patient-reported outcomes, certainly useful for a more comprehensive analysis of the effectiveness of treatments, to improve their quality also in the direction of the personalization of medicine already hoped for by Umberto Veronesi. Currently, the new regulation of the EHR has not intervened on the purposes of clinical governance and research that can be pursued with this tool, which are still governed by the Prime Ministerial Decree No. 178 of 2015, which should therefore be amended in this regard.

Naturally, as emphasized in the Authority's opinion of June 2023 on the EHR, and even more so where there is an intention to extend its purposes, it is necessary to coordinate its regulation with the implementing decrees concerning digital health and, in particular, with the one aimed at regulating the Health Data Ecosystem, as provided for by Article 12, paragraph 15-quater, of Legislative Decree No. 179 of 2012. Only in this way can a comprehensive coherence of the information systems provided be ensured and their rational complementarity, also through interoperability measures tailored to the safeguards established by the Authority.

Similarly, the more systematic use (as hoped for in the Resolutions) of disease registries for clinical governance and research purposes can occur by extending their application scope with specific regulatory amendments to be submitted, in this case as well, to the opinion of the Authority as prescribed by Article 36, paragraph 4, of Regulation 2016/679, in order to ensure that this occurs in compliance with the guarantees established by the relevant regulation.

Overall, the commitment to the Government, contained in the guiding documents, for the realization of an institutional communication and awareness campaign on the importance of managing health data in a manner compliant with data protection guarantees, can only find agreement with the Authority, which has intervened several times in this sense: especially during the pandemic but also recently, regarding the new frontiers of e-health, such as the use of AI.

Finally, regarding the commitment proposed by the Resolutions to the Government, in support of the

need for the timely approval of the European Regulation on the European Health Data Space, the evolution of the text following the approval on December 13 of several amendments that move in the direction outlined by the Authority and the European Data Protection Committee, in their joint opinion No. 3 of 2022, is positively noted.

In particular, the provision of greater guarantees (in line with those provided by the Data Governance Act) regarding the altruism of health data (a beautiful example of the use, for solidarity purposes, of personal data of such delicacy) and the use of automated decisions deserves appreciation; the introduction of opt-in for the secondary use of certain specific health data, such as genomic data or data from wellness applications, and, in general, the opt-out for the secondary use of data; greater guarantees for the interoperability of wellness applications with electronic health record systems; specific protections for so-called group privacy, that is, specific prohibitions on the discriminatory use of health data against individuals or groups of people, even regarding job offers or contractual conditions.

The evolution of the text itself demonstrates the awareness of how the synergy between innovation, health governance, and data protection will play an increasingly decisive role for our societies, a challenge we must commit to winning, once again in the name of the centrality of the person and their dignity: those bonds that, as Aldo Moro explained in the Constituent Assembly, even the collective interest in public health cannot override.